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June 16, 2025

Dr. Thomas Keane
Assistant Secretary for Technology Policy, Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Centers for Medicare & Medicaid Services

RE: Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Dear Dr. Keane,

On behalf of the Advanced Medical Technology Association (AdvaMed), we are pleased to offer comments on the Request for Information on the health technology ecosystem, published in the *Federal Register* on May 16, 2025 (CMS-0042-NC). AdvaMed member companies produce the medical devices, diagnostic products, and health information systems that are transforming health care through earlier disease detection, less invasive procedures, and more effective treatments. AdvaMed members range from the largest to the smallest medical technology innovators and companies. We are committed to ensuring patient access to lifesaving and life-enhancing devices and other advanced medical technologies in the most appropriate settings.

Leaders of this Administration have been outspoken on their concern for poor health outcomes in the United States, despite the abundant resources at our disposal. We at AdvaMed and across the medtech community share this deep concern. We also know medtech is a proven part of the solution. Medtech works. But it works only if patients and their trusted health care providers have access to it. Access is complicated by many factors: lack of insurance coverage of medtech-enabled procedures; lack of health insurance generally; inadequate provider reimbursement; limited access to specialists and care due to factors including geography and health care provider shortages; limited understanding of available treatment options; and high copayments and other out-of-pocket expenses associated with health care.

These complications to access are seen clearly through the lens of advancements in health technology, including the rapid expansion of artificial intelligence, remote patient monitoring, and other digital health technologies. Further, critical data, especially from connected medical devices, is siloed across EHRs, payer systems, and proprietary platforms, and patients and providers often lack the ability to access a complete, longitudinal view of care. In that vein, we appreciate this request for information and CMS' efforts to advance interoperability and a more connected healthcare ecosystem.

A. Definitions of Terms Used in This RFI

Digital Tools and Digital Health Products

While we appreciate and support the Administration's goal to expand patient engagement in their care through the use of digital tools and digital health products, we would caution against the overly broad definitions of these terms as provided in this Request for Information. There is a broad array of wellness technologies available to Americans today, but only a small portion of those have undertaken the rigorous requirements for Food and Drug Administration regulation. This regulatory framework ensures patients can rely on the digital health tool or product to perform as intended, maintain appropriate security protocols, and produce clinically valid data. ***We therefore urge the agency to make clear delineations between the use of regulated and unregulated digital health technologies moving forward.***

B. Patients and Caregivers

PC-4

While the primary focus of this section is to enhance access to medical data for patients and caregivers, it is equally important to reflect on scenarios where providers may need to restrict immediate access to certain medical information present in the systems. Sensitive data, such as cancer diagnoses or HIV test result information, should be handled with care and communicated by trained professionals to prevent undue stress and panic. These workflows should be coded in the patient/caregiver-focused apps and services.

PC-5

CMS should not be granted additional authority to review or approve digital health products beyond its existing role in reimbursement processes (i.e., coverage, coding, and payment). The FDA already has a robust regulatory framework in place to evaluate and authorize regulated digital health products, ensuring they meet rigorous standards for safety, efficacy, and quality. Introducing a separate review process by CMS to assess these same elements (or additional elements, beyond current reimbursement standards) would create unnecessary duplication, introduce inefficiencies, and risk delaying patient access to innovative technologies. ***Rather than duplicating efforts, CMS and FDA should prioritize continued collaboration to support timely access to FDA-authorized digital health solutions while preserving the distinct and complementary roles of each agency.***

PC-13

As part of ONC's effort to engage patients and caregivers in identifying and reporting information blocking, we encourage the agency to increase patient education on their rights and data ownership. For example, ownership of health data/PHI collected by non-FDA cleared wearables could fall with the patient or the manufacturer, depending upon the product. Improving patient education around data ownership can help them make informed decisions about which products to use in managing their health and, by extension, proactively engaging in the management of the resulting data. This level of empowerment in data ownership may also help patients engage their providers in condition management and tracking patient-specific health outcomes. CMS should also ensure data access remains unrestricted as patients move across systems and penalize those who engage in information blocking.

C. Providers

PR-1 and PR-2

In considering what CMS and its partners can do to encourage providers to leverage approved digital health products for their patients, we encourage the agency to consider the varying and intersecting use of digital health products across settings of care (i.e., inpatient, outpatient, and home) as well as particular patient populations. By improving interoperability of provider and patient technologies, the agency can facilitate patients' care journey across settings and simplify providers' patient management processes. Ensuring that necessary reimbursement mechanisms – including adequate payments and clear coding frameworks – are in place across the respective fee-for-service payment systems for these settings is also essential for patient access. This is particularly important when patients are discharged home with ongoing health management and monitoring needs. ***We therefore encourage the agency to revisit policies impacting patients' ability to access digital health tools in the home setting, such as remote patient monitoring technologies, and to remove all barriers impeding their use when medically necessary.***

PR-3

The current siloed nature of data poses significant challenges to access because it is essential for providers to view comprehensive patient medical data, regardless of storage format, to make informed medical decisions. Also, data should not be presented just in raw, unstructured formats; ideally, it should be accompanied by an AI-driven front-end layer that enables the derivation of actionable insights (in addition to raw data), thereby facilitating better clinical decision-making. ***Ensuring providers have access to all relevant data enhances their understanding of diseases and conditions, ultimately enabling them to deliver improved care to subsequent patients.***

E. Technology Vendors, Data Providers, and Networks

TD-1

Increasing access to data is central to developing digital health tools and digital health products. For many years, medtech companies have used patient-level data to personalize care and ensure each patient is provided with the proper plan and execution specific to their needs. As technology has evolved, medtech companies have been able to use large data sets for model development that helps optimize patient care by allowing for "virtual twins" that can help predict a patient's outcomes based on similar patients. And moving forward, access to unstructured data can help researchers identify new interventions to improve patient care. Taken together, these uses of data improve patient outcomes, quality of care, safety, and economic efficiency in the health care system. We therefore recommend the agency collaborate with a range of stakeholders, including providers, hospitals, and medtech manufacturers, to understand how modifying existing interoperability requirements can expand the digital health products available to Medicare beneficiaries. As part of this engagement, we encourage the agency to consider:

- As discussed further under VB-1 below, engaging in cross-agency collaboration to bring innovations to Medicare population more quickly;

- Ensuring access to high-quality patient data that is standardized and contextually rich, such as linking diagnostics with treatment and outcomes;
- Considering methods for deidentifying medical images so they can be handled as regular PHI rather than identifiers; and
- Developing and improving standards for device-to-device exchange of data.

Additionally, in order for developers to build digital health products for Medicare beneficiaries, reimbursement mechanisms must be modernized so that commercialization of medically necessary tools is viable. ***CMS should review current coverage, coding, and payment policies to identify barriers to patient access (e.g., lack of clear and predictable payment pathways for algorithms and other software-based interventions furnished by hospitals or used in the home setting under the current definition of durable medical equipment). CMS should also consider shifting the approval and reimbursement paradigm for digital products to prioritize ongoing post-market surveillance data over initial clinical evidence requirements.***

TD-2

CMS previously finalized adoption of a series of application programming interfaces (APIs), including a series of APIs related to the prior authorization process and patient and provider access to data under these APIs.[1] AdvaMed is broadly supportive of CMS' requirements for enhancing patient access to prior authorization-related data under its APIs, as we believe these APIs can enable patients to participate in their care more and reduce administrative burden on both providers and payers. AdvaMed strongly supports efforts to educate patients and their caregivers about the availability and utility of these APIs, as well as ongoing efforts to improve access to these technologies and digital literacy generally.

Looking forward, we encourage CMS to explore additional strategies to promote access to timely prior authorization-related information for patients who are uncomfortable with or unable to use the underlying technology. CMS' finalized policies address only the role of the payer and provider in collecting and sharing data about a patient's care. While most prior authorization requests are submitted by physicians' offices and medical facilities, some health care providers may utilize third parties, such as medical device manufacturers, for prior authorization support services. Manufacturers and providers have standard processes in place to collect the required data in a HIPAA-compliant manner to assist patients in accessing the treatment recommended by their physicians, while reducing the inefficient use of provider and staff time. Medical device manufacturers offering prior authorization support services are dedicated to providing transparency regarding their role in the submission process while interacting with the patient's payer. We recognize that as new, innovative medical devices continue to be developed, prior authorization support services will be needed to assist providers who seek to use these devices to meet their patients' needs. ***We therefore request CMS provide a mechanism for medical device manufacturers and other third-party submitters to utilize CMS' patient access and prior authorization APIs to submit prior authorization requests directly on behalf of providers.***

[1] These APIs are the Patient Access API and Prior Authorization Requirements, Documentation, and Decision API. These APIs are currently slated for implementation beginning January 1, 2026.

TD-3

We are encouraged by the Administration's willingness to adopt federated identity providers and would advocate for an identity provider certification process to ensure trust and security in the ecosystem. Federated identity providers have been widely implemented in many industries and have been found to simplify a user's login experience and improve the overall application security. ***However, given the sensitive nature of health data, certain standards and requirements should be considered for identity providers.***

TD-7

USCDI plays a vital role in the health data ecosystem. We are encouraged by the evolution of the standard and the actions CMS and ASTP have taken to promote adoption. With the expansion of additional data elements and supporting technology standards, USCDI can benefit the total product lifecycle of medical devices. One example is the recent development of USCDI+ and the inclusion of use cases related to registry data submission, clinical trial matching, and adverse events for cancer. We also recognize that interoperability standards for oncology are more mature than other clinical domains, and ***we encourage additional investments to develop similar interoperability standards in other clinical domains so that they can be considered for USCDI/USCDI+ inclusion.***

TD-10

In addition to previous steps taken by ONC to implement the 21st Century Cures Act, we recommend CMS consider development of a FHIR-based API connecter for any EHR and other developers subject to the ONC Health IT Certification Program. Healthcare systems often use different data models and terminology, which can make transferring and translating data from one system to another time-consuming and prone to mistakes. ***Implementing a FHIR-based API connector would ensure more seamless exchange of health information, promote interoperability, and improve patient care coordination.***

F. Value-Based Care Organizations

VB-1

In considering incentives to encourage ACOs to leverage digital health products more effectively with their patients, we encourage the agency to focus on increasing connectivity across different components of the federal healthcare system. For example, the Advanced Research Projects Agency for Health (ARPA-H) has provided research funding for a variety of AI- and digital-enabled solutions for major biomedical concerns. This funding is provided, in part, to streamline widespread access to and use of these technologies. ***We encourage CMS to engage with ARPA-H in reviewing these grants and their associated technologies to determine whether they could be incorporated into ACOs or other value-based case paradigms, such as an Innovation Center model, to facilitate Medicare beneficiary access to these cutting-edge technologies.***

We appreciate the opportunity to provide feedback as the agency considers changes to its engagement in the health technology ecosystem. If you have any questions, please contact Kirsten Tullia (ktullia@advamed.org).

Sincerely,

Kirsten Tullia, JD, MPH

Senior Vice President, Payment and Health Care Delivery Policy

AdvaMed